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Topic 6

Understanding stress, depression and anxiety

The Wellbeing Toolkit

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Aims of session

- To clarify the nature of depression and anxiety
- To highlight some key tools and effective thinking strategies from Cognitive behaviour therapy (CBT) and Positive psychology approaches which are useful in supporting CYP
- To introduce some of the resources which support the development of these skills
- To ensure that practitioners have the opportunity to develop and practice their own skills and access resources which are practical and user friendly

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Using the word ‘stressed’

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The loss of sadness?

- For the last 2,500 years human beings could distinguish between normal sadness or suffering and clinical depression.
- Since the development of the classification system for mental illness the boundary has become blurred
- The only context the manual takes into account for depression is bereavement

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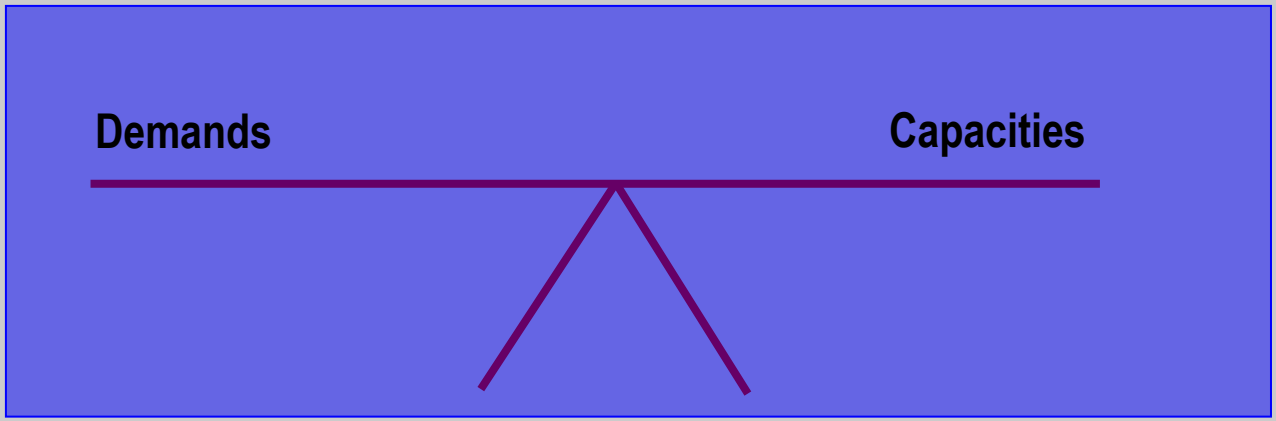
What is stress?

"Stress is a demand made upon the adaptive capacities of the mind and body".

Fontana, 1989, p.3



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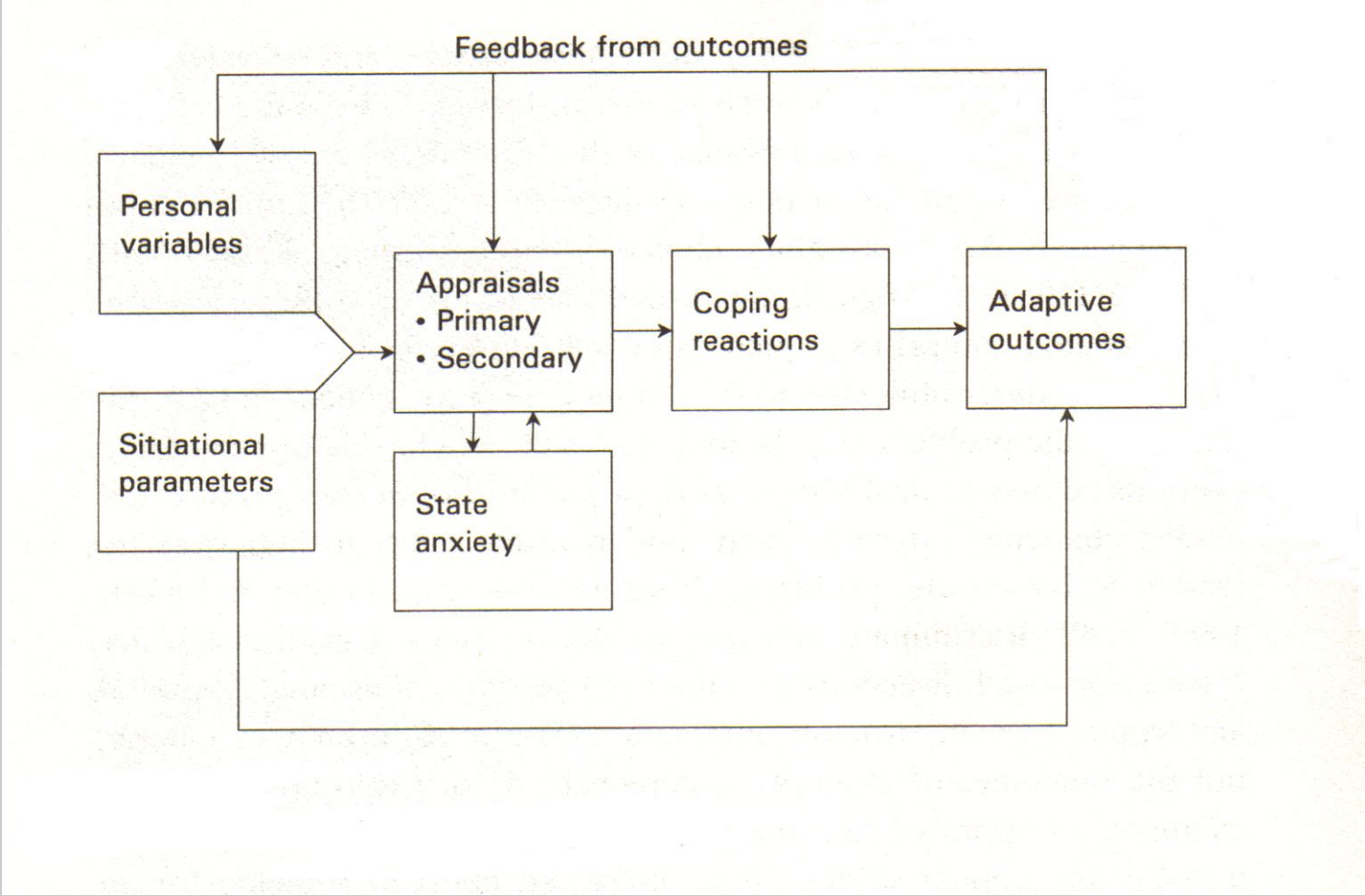
A transactional model of stress

From Matthews, Zeidner and Roberts, 2002



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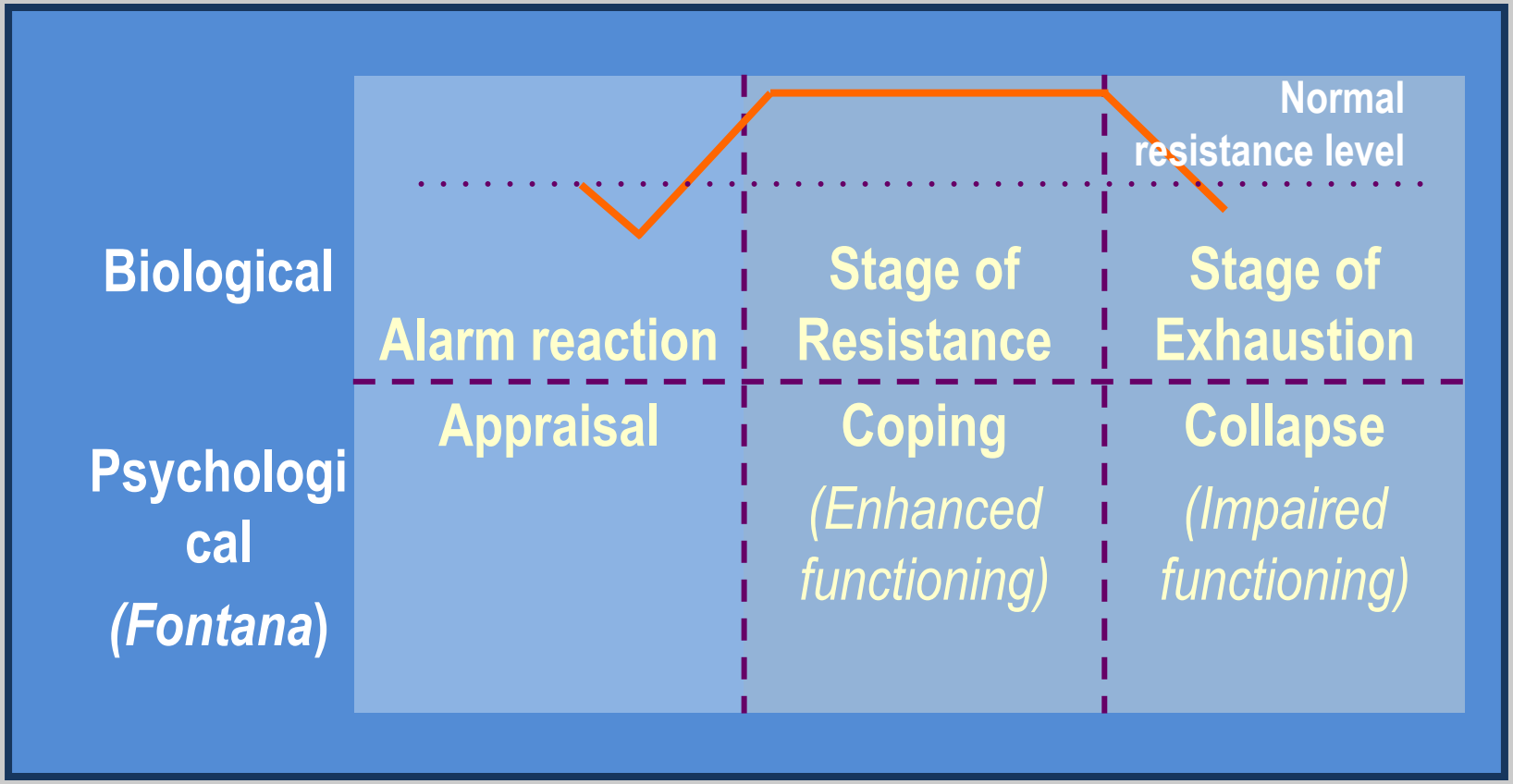
Reaction to Stressors

General Adaption Syndrome Model



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Effects of Too Much Stress I

Cognitive Effects

- Concentration and attention span decrease
- Distractibility increases
- Short- and long-term memory deteriorate
- Response speed becomes unpredictable
- Error rate increases
- Organisation and planning deteriorate
- Delusions and thought disorders increase

Emotional Effects

- Ability to switch off worry and anxiety reduces
- Feelings of health and well-being reduce
- Over-sensitivity, defensiveness and hostility more likely
- Emotional outbursts can increase
- Depression and helplessness more likely
- Self-esteem falls

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Effects of Too Much Stress II

Behavioural Effects

- Speech problems increase
- Interests and enthusiasms diminish
- Absenteeism increases
- Drug abuse increases
- Low energy levels
- Disrupted sleep patterns
- Increased cynicism about clients and colleagues
- New information is ignored
- Responsibilities are shifted on to others
- Problems are solved at an increasingly superficial level
- Bizarre behaviour patterns appear
- Suicide threats may be made

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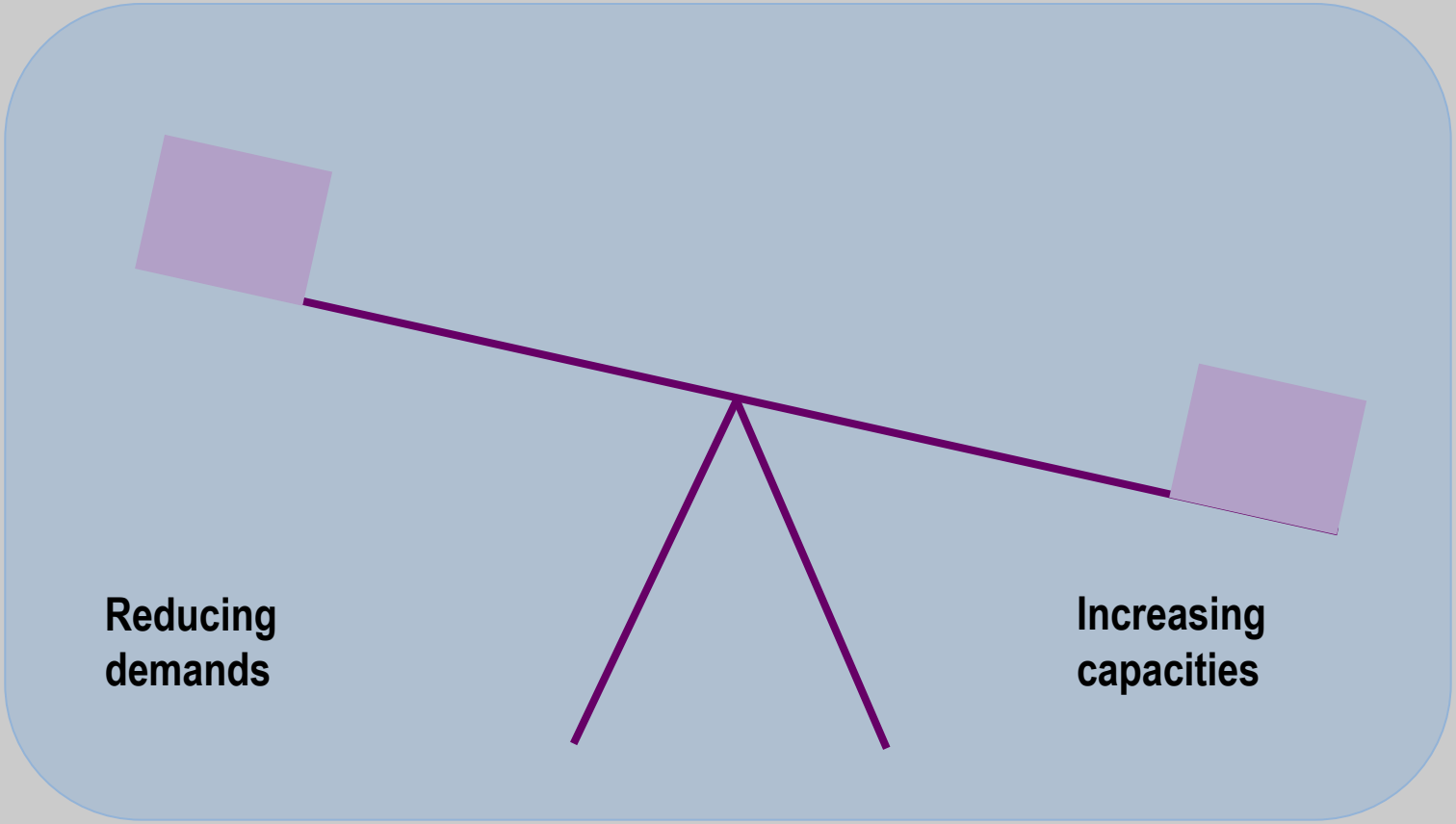
Managing Stress



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Sources of Stress

(Fontans, 1989)

Work

- Unclear role
- Powerlessness
- Time pressures
- Isolation from Colleagues support
- Inadequate leadership
- Role conflict
- Overwork
- Lack of variety
- Poor communication
- Interpersonal conflicts

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Sources of Stress cont.

Home

- Partner
- Children
- Domestic arrangements
- Environmental pressures

Personal

- Unrealistically high expectations and perfection issues
- Negative cognitive appraisals / self talk
- Need to please / be liked
- Temperament
- Low assertiveness

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Sources of Stress

	Familiar	New
WORK		
HOME		
YOU		

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Increasing capacities

(See Wright & Frederickson, 1992)

- Healthy lifestyle
- Relaxation
- Assertiveness
- Effective thinking
- Time management
- Enhancing study skills

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What is depression?

We often use the expression ‘I feel depressed’ when we’re feeling sad or miserable about life. Usually, these feelings pass in due course. But, if the feelings are interfering with your life and don't go away after a couple of weeks, or if they come back, over and over again, for a few days at a time, it could be a sign that you're depressed in the medical sense of the term.

And

In its mildest form, depression can mean just being in low spirits. It doesn’t stop you leading your normal life, but makes everything harder to do and seem less worthwhile. At its most severe, major depression (clinical depression) can be life-threatening, because it can make you feel suicidal or simply give up the will to live.

Different types

- **Seasonal affective disorder (SAD)** – this is seasonal depression which is related to day length. It usually comes on in the autumn and winter, when days are short and the sun is low in the sky, and gets better as the days get longer and brighter.
- **Postnatal depression** – many mothers have ‘the baby blues’ soon after the birth of their baby, but it usually passes after a day or two. Postnatal depression is a much more serious problem and can occur any time between two weeks and two years after the birth.
- **Bipolar disorder (manic depression)** – some people have major mood swings, when periods of depression alternate with periods of mania. When manic, they are in a state of high excitement, and may plan and may try to carry out over-ambitious schemes and ideas. They often then have periods of severe depression.

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The numbers

At any one time, the estimated number of children and young people suffering from depression:

- 1 in 100 children
- 1 in 33 young people

Prevalence figures exceed treatment numbers:

- about 25% of children and young people with depression detected and treated

Suicide is the:

- 3rd leading cause of death in 15–24-year-olds
- 6th leading cause of death in 5–14-year-olds

Symptoms

Key symptoms

- persistent sadness, or low or irritable mood
- loss of interests and/or pleasure
- fatigue or low energy

Associated symptoms

- poor or increased sleep
- low self-confidence
- poor concentration or indecisiveness
- poor or increased appetite
- suicidal thoughts or acts
- guilt or self-blame
- agitation or slowing of movement

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Our job

Professionals in primary care, schools and community need to:

- be aware of risk factors
- engage in ‘active listening’ and ‘conversational techniques’
- detect symptoms
- provide appropriate support
- know when to refer

A holistic approach

Care should be comprehensive and holistic and take into account:

- drug and alcohol misuse
- experience of bullying or abuse
- parental depression
- risks of self-harm and suicide
- use of self-help materials and methods
- issues of confidentiality

Mild depression

Treatment includes:

- up to 4 weeks ‘watchful waiting’
- non-directive supportive therapy
- group CBT
- guided self-help
- no use of antidepressants at this stage

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Moderate to severe depression NICE guidance

General recommendations

- Approach tailored to needs of family
- Family's preferences to be taken into account
- E.g. when too depressed
- Does not want family involved
- May require change of approach especially if symptoms deteriorate

Treatment starts with review by multidisciplinary team

First line of treatment is specific psychological therapy for about 3 months

- Individual cognitive behavioural therapy
- Interpersonal therapy
- Shorter-term family therapy

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Facts about anxiety

- Anxiety disorders are among the most common mental, emotional, and behavioural problems to occur
- About 13 of every 100 children and adolescents ages 9 to 17 experience some kind of anxiety disorder
- Girls are affected more than boys. About 50% of children and adolescents with anxiety disorders have other mental/behavioral disorder
- Anxiety disorders may coexist with physical health conditions as well

Brief definition

- Anxiety is a general feeling of apprehension or worry and is a normal reaction to stressful situations
- Red flags should go up when the feelings become excessive, thoughts become irrational and everyday functioning is debilitated
- Anxiety disorders are characterized by excessive feelings of panic, fear, or irrational discomfort in everyday situations

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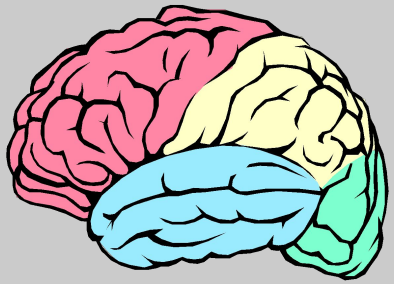
Production of fear and anxiety

Using brain imaging and neurochemical techniques several parts of the brain have been identified as key in the production of fear and anxiety

Two main components involved are the amygdala and the hippocampus

- **Amygdala** - Emotional memories are stored here and alerts brain that a threat is present
- **Hippocampus** - Encodes specific threatening events into memories



How anxiety is manifested

- Students may feel a sense of dread
- Have fears of impending doom
- Experience a sense of suffocation
- Anticipation of unarticulated catastrophe
- Loss of control over their breath, swallowing, speech, and coordination
- Somatic Complaints

Types of anxiety disorders

Generalized Anxiety Disorder (GAD)
GAD results in students experiencing six months or more of persistent, irrational and extreme worry, causing insomnia, headaches, and irritability.

Post-Traumatic Stress Disorder (PTSD)
PTSD can follow an exposure to a traumatic event such as natural disasters, sexual or physical assaults, or the death of a loved one. Three main symptoms: reliving of the traumatic event, avoidance behaviours and emotional numbing, and physiological arousal such as difficulty sleeping, irritability or poor concentration.

Panic Disorders

Characterized by unpredictable panic attacks, which are episodes of intense fear, physiological arousal, and escape behaviours. Common symptoms: heart palpitations, shortness of breath, dizziness and anxiety and these symptoms are often confused with those of a heart attack.

Specific Phobias

Intense fear reaction to a specific object or situation (such as spiders, dogs, or heights) which often leads to avoidance behaviour. The level of fear is usually inappropriate to the situation and is recognized by the sufferer as being irrational

Disorders continued

Social Phobia

Extreme anxiety about being judged by others or behaving in a way that might cause embarrassment or ridicule and may lead to avoidance behaviour.

Separation Anxiety Disorder

Intense anxiety associated with being away from caregivers, results in youths clinging to parents or refusing to do daily activities such as going to school.

Obsessive-Compulsive Disorder (OCD)

Students may be plagued by persistent, recurring thoughts (obsessions) and engage in compulsive ritualistic behaviours in order to reduce the anxiety associated with these obsessions (e.g. constant hand washing).

Comorbidity

Comorbid diagnoses of depressive disorders, ADHD, and other anxiety disorders are common in anxiety patients.

Symptoms that may appear to be ADHD:

- Restlessness, feeling keyed up or on edge
- Difficulty concentrating, mind going blank
- Irritability
- Clinically significant distress or impairment in social or academic areas

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Effective ways to treat anxiety

- Cognitive-behavioural treatment(young people learn to deal with fears by modifying the ways they think and behave)
- Relaxation techniques
- Biofeedback (to control stress and muscle tension)
- Family therapy
- Parent training
- Medication

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Strategies for dealing with anxious Students

- Because transitions and separation are frequently difficult for children with anxiety disorders, accommodate student's late arrival and provide extra time for changing activities and locations.
- Recognize that often it is a child's anxiety that causes him or her to disregard directions, rather than an intentional desire to be oppositional.
- Develop a "safe" place where the child can go to relieve anxiety during stressful times or provide calming activities.
- Encourage the development of relaxation techniques that can work in the school setting. Often these can be adapted from those that are effective at home.

- Work with a child regarding class participation and answering questions on the board, understanding that many anxious CYP fear answering incorrectly.
- Encourage small group interactions and provide assistance in increasing competency and developing peer relationships.
- Reward the student's efforts.
- Provide an organized, calming, nurturing and supportive environment.
- For maximum effectiveness, foster feedback from CYP about these interventions

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Need to develop school based preventative strategies

- Healthy lifestyle
- Relaxation
- Assertiveness
- Effective thinking – CBT
- Time management
- Enhancing study skills

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Effective thinking

Identifying and challenging irrational beliefs is an important first step. Such beliefs are exaggerated, oversimplified, illogical, unrelated to evidence or too absolute.

- When I make a mistake it is terrible, catastrophic, disastrous
- If someone gets upset when I say something to them, then it is all my fault
- If someone turns down my request, they are rejecting me
- It is vital that everyone approves of me and what I do all the time (even if I am totally irritating and unreasonable)

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Effective thinking

Irrational belief

- I need to be thoroughly competent and achieving or I am not worthwhile
- My past is the cause of my present problems

Rational equivalent

- While I gain great pride from competence and achievement, I need to accept that I, like everyone else, have human limitations and fallibilities
- Just because past events have strongly influenced me, I do not have to be attached to them for ever, I can learn from them

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Ineffective self talk

- Worrying about a future situation
- Catastrophising
- Setting unrealistic standards
- Over-reacting to feelings of anxiety
- Being overly worried about what others think
- Putting yourself down
- Focusing on set-backs and not owning successes
- Not setting clear, manageable goals or focusing on tasks that will achieve them.

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Constructive Self Talk

Calming Self-Talk

- Tell yourself to stay calm – ‘relax, calm down, take a deep breath, count to 10, breath slowly…’
- Tell yourself you can cope – ‘I can handle this situation. I’ve got the skills to cope with this. I can solve this problem if I relax and think about it.’

Coaching Self-Talk

- This might involve using a problem solving framework – ‘Right! First define an objective … now think of possible solutions … now choose one to implement’.
- Or reminding yourself to use new skills in tricky situations – ‘Right! She’s getting angry. I must listen to what she is saying and acknowledge her feelings’.

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MOST IMPORTANT TO TEACH THESE SKILLS TO THE MOST VULNERABLE STUDENTS

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All learners will probably experience the associated stressors when revising/undertaking assessments:

- Subjective - aggression, fatigue, low self-esteem
- Behavioural - restlessness, emotional outbursts, taking drugs
- Cognitive - inability to concentrate, mental blocks, hypersensitive
- Physiological - sweating, dry mouth, increased heart rate

(Cox, 1981)

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It's all about control

"People who believe they can exercise control do not engage in apprehensive thinking, they are not bothered by threat or challenge"

(Frydenberg, 1997)

Need to promote:

Autonomy and control

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Planning ahead strategy helps to achieve a sense of control

1. **Imagine** - situation in detail
2. **Prepare** - seek support
3. **Be aware** of all possibilities and tell MIND to stay calm in each one
4. **Make contingency plans** - e.g. make notes for your presentation!
5. **Relax** - genuine relaxation before the event
6. **Unwind** - reward yourself

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Implementing change – behavioural experiments

Stages

1. Identify the unhelpful belief
2. identify the more helpful alternative
3. Test which belief is more accurate
4. Discuss the results of the test

Outcome

Evidence to challenge the core beliefs and NATs

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More strategies to implement change

Re-Framing

- Re-frame NATs into more balanced, positive, realistic thoughts.

Distraction

- Describing what you see.
- Get absorbed
- Self-talk
- Bin them
- Top talk

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And more

Positive Automatic Thoughts

- Weekly Positive Diary, for example: ‘Someone said hello.’
  ‘My teacher praised me.’

Goal Setting

- Set realistic goals
- Practise strategies

Prevention is better than cure

Logical conclusion

- provide copious opportunities to learn and practice a range of effective self management skills
- increase ability to cope with stress and anxiety
- maintain locus of control and well-being

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Teach all students methods of coping with stress and anxiety

- Diet
- Relaxation
- Meditation
- Time management
- Physical exercise
- Additional training
- Counselling
- Reframing
- Improving social support networks
- Peer support groups
- Improved resource allocation
- Self-analysis

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Thank you for listening and participating